

1. Administrative & Study Identification

**Full Study Title:** Study to Compare the Efficacy and Safety of Olaparib When Given in Combination With Carboplatin and Paclitaxel, Compared With Carboplatin and Paclitaxel in Patients With Advanced Ovarian Cancer **Short Title/Acronym:** Study 41 / Olaparib in Advanced Ovarian Cancer **Protocol Number:** D0810C00041 **NCT Number:** NCT01081951 **Sponsor/Company Name:** AstraZeneca **Investigational Product:** Olaparib (AZD2281 / Lynparza) **Phase of Development:** Phase II (PHASE2) **Trial Status:** ACTIVE\_NOT\_RECRUITING **Medical Monitor:** AstraZeneca Clinical Operations Lead

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2. Study Synopsis & Rationale

**2.1 Study Objective Primary Objective:** To compare the efficacy, specifically Progression-Free Survival (PFS), of olaparib in combination with paclitaxel and carboplatin followed by olaparib maintenance monotherapy versus standard paclitaxel and carboplatin alone in patients with advanced ovarian cancer. **Secondary Objectives:** To evaluate Overall Survival (OS), Objective Response Rate (ORR), percentage change in tumor size, efficacy by BRCA1/2 mutation status, and the safety and tolerability of the combination regimen.

**2.2 Background & Rationale** The poly(ADP-ribose) polymerase (PARP) inhibitor olaparib has demonstrated antitumour activity in patients with platinum-sensitive, recurrent, high-grade serous ovarian cancer, particularly those harboring BRCA mutations. This study aims to assess whether the concurrent administration of olaparib with standard platinum-based chemotherapy (carboplatin and paclitaxel), followed by continuous olaparib maintenance therapy, can significantly improve progression-free survival without yielding unacceptable overlapping toxicities, compared to chemotherapy alone.

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3. Study Design & Methodology

**3.1 Design Framework Study Type:** Interventional **Control Type:** Active Comparator (Standard Chemotherapy) **Blinding:** Open-label **Randomization:** 1:1, Stratified (by the number of previous platinum-containing regimens received [1 or >1] and time to disease progression following the previous platinum regimen [>6 to ≤12 months or >12 months]).

**3.2 Planned Enrollment & Duration Targeted Enrollment:** 162 randomized patients **Number of Sites:** 43 investigational sites across 12 countries **Estimated Study Start:** 12-Feb-2010 **Status:** ACTIVE\_NOT\_RECRUITING (Active but not currently recruiting)

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## 4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Adult females ( $\geq 18$  years of age). 2. Confirmed diagnosis of platinum-sensitive, recurrent, high-grade serous ovarian cancer. 3. Received up to three previous courses of platinum-based chemotherapy and were progression-free for at least 6 months prior to randomization (following the end of the last platinum treatment). 4. At least one lesion that is suitable for accurate repeated measurements.

**4.2 Exclusion Criteria** 1. Prior exposure to any PARP inhibitor or investigational targeted therapies for ovarian cancer. Patients receiving any systemic anticancer chemotherapy or radiotherapy (except palliative) within two weeks from the last dose prior to study treatment. 2. Hypersensitivity to pre-medications required for treatment with paclitaxel/carboplatin. 3. Inadequate hematologic, hepatic, or renal function preventing safe administration of chemotherapy. 4. Significant cardiovascular disease or concurrent malignancies with a high risk of progression/death.

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## 5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** \* **Combination Phase:** Olaparib 200 mg capsules twice daily (BID) orally on Days 1–10 of each 21-day cycle, plus Paclitaxel ( $175 \text{ mg/m}^2$  IV on Day 1) and Carboplatin (Trade Name: Paraplatin, ATC Code: L01XA02) (AUC 4 IV on Day 1). \* **Maintenance Phase:** Olaparib monotherapy 400 mg capsules twice daily (BID) given continuously. \* **Comparator Arm:** Paclitaxel ( $175 \text{ mg/m}^2$  IV on Day 1) plus Carboplatin (AUC 6 IV on Day 1) every 21 days, followed by no further maintenance treatment. **Route of Administration:** Oral (Olaparib) and Intravenous (Paclitaxel/Carboplatin). **Duration of Treatment:** Cycles are 21 days long; treatment continues until objective disease progression or unacceptable toxicity.

**5.2 Concomitant & Prohibited Therapy Permitted:** Standard antiemetics, hypersensitivity prophylaxis (required for paclitaxel), and supportive care medications. **Prohibited:** Concomitant use of other anticancer agents, investigational drugs, or potent CYP3A4 inhibitors/inducers that may interact with olaparib pharmacokinetics.

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## 6. Study Timeline & Visit Schedule

| Visit ID     | Window (Days)         | Key Activities/Procedures                                                      |
|--------------|-----------------------|--------------------------------------------------------------------------------|
| Screening    | Day -28 to 0          | Informed Consent, Medical History, Baseline Scans (CT/MRI), Labs, BRCA testing |
| Baseline     | Day 0                 | Randomization, First Dose of Chemotherapy and Olaparib                         |
| Treatment    | Day 1 of 21-Day Cycle | Chemotherapy infusion, Olaparib dispensing, Safety Labs, AE monitoring         |
| Tumor Eval   | Week 9 and Q9W        | Efficacy Assessment via CT/MRI for RECIST v1.1 evaluation                      |
| End of Study | At Progression        | Final Vital Signs, Exit Interview, IP Accountability, Survival Follow-up       |

## 7. Outcome Measures (Endpoints)

**7.1 Primary Endpoint Definition:** Progression-Free Survival (PFS) (based on independent central review), defined as the time from randomisation until objective disease progression as defined by Response Evaluation Criteria In Solid Tumours (RECIST) v1.1 (≥ 20% increase in the sum of diameters of target lesions from minimum, clinically significant progression in non-target lesions, or the presence of a new lesion) or death (by any cause in the absence of progression). **Measurement Method:** Independent central review of radiological imaging.

**7.2 Secondary & Exploratory Endpoints**

- Overall Survival (OS).
- Percentage Change in Tumour Size (calculated at Week 9) and Objective Response Rate (ORR).
- Efficacy analysis stratified by retrospective BRCA1/2 mutation status and homologous recombination deficiency (HRD).

## 8. Safety & Adverse Event Reporting

**Adverse Event (AE) Monitoring:** Routine collection via patient interview, physical examination, and clinical laboratory testing at each visit, graded per standard CTCAE criteria. **Serious Adverse Events (SAEs):** Life-threatening events or events requiring hospitalization reported within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Independent internal/external committee designated to review safety and tolerability, notably managing any overlapping hematologic toxicities.

## 9. Statistical Analysis Plan (SAP) Summary

**Analysis Populations:** Intent-to-Treat (ITT) population utilized for primary progression-free survival efficacy analysis. Safety Set used for all AE/SAE tabulations. **Sample Size Justification:** Powered to detect a statistically significant hazard ratio improvement in PFS for the investigational combined arm versus

standard doublet chemotherapy. **Handling of Missing Data:** Imputations were strictly used for missing tumor size data where possible to assess percentage change at week 9; standard right-censoring was applied for survival endpoints for patients without an event at the data cutoff.

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## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

**Monitoring Plan:** Scheduled onsite and remote monitoring visits to evaluate source data verification and protocol compliance. **Audit**

**Trail:** Mandatory eCRF tracking, query generation, and central imaging review independent from the local investigator to ensure objective assessment of progression.

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## 11. Study Identifiers & Governance

**Posting ID:** 1065164384493657566 **Primary ID:** D0810C00041 **Registry Number:** NCT01081951 **Sponsor/Lead Organization:** AstraZeneca **Trial Status:** ACTIVE\_NOT\_RECRUITING **Study Title:** Study to Compare the Efficacy and Safety of Olaparib When Given in Combination With Carboplatin and Paclitaxel, Compared With Carboplatin and Paclitaxel in Patients With Advanced Ovarian Cancer **Official Title:** A Phase II Open Label Randomised Comparative Multicentre Study to Compare the Efficacy and Tolerability of Olaparib in Combination With Paclitaxel and Carboplatin Versus Paclitaxel and Carboplatin Alone in Patients With Platinum Sensitive Advanced Serous Ovarian Cancer

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## 12. Clinical Framework (Ovarian Cancer Specific)

**Disease Area:** Ovarian Cancer **Preferred Name:** Malignant neoplasm of ovary **Preferred UMLS Name:** Ovarian Neoplasms **MeSH Code:** D010051 **SNOMED ID:** 363443007 **HPO Code:** HP:0100615 **Primary Condition:** Recurrent, Platinum-Sensitive High-Grade Serous Ovarian Cancer **Diagnosis Threshold:** Histologically/cytologically confirmed recurrence with a platinum-free interval  $\geq$  6 months. **Medical Methodology:** Platinum-based doublet chemotherapy overlaid with PARP-inhibition and subsequent targeted maintenance. **Optimization Target:** Maximization of Progression-Free Survival (PFS) and extending the platinum-free interval.

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## 13. Study Procedures & Methodology

**Management Pathway:** Systemic chemotherapy combined with oral targeted therapy. **Education Protocol (HCP):** Training on protocol-specific dosing reductions, specifically omitting olaparib

maintenance or reducing chemotherapy AUC/doses in the presence of severe myelosuppression. **Education Protocol (Patient):** Self-administration of olaparib on Days 1-10 of the chemotherapy cycle; maintenance of continuous dosing post-chemo, fasting requirements, and safety reporting. **Quality Audit Mechanism:** Centralized blinded imaging review to audit local investigator RECIST v1.1 measurements.

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**14. Observation & Follow-Up Schedule**

**Total Duration:** Continuous until objective disease progression or unacceptable toxicity. **Onsite Visit Cadence:** Every 3 weeks (Day 1 of each 21-day cycle) during both combination and maintenance phases. **Remote Monitoring Frequency:** Inter-cycle as clinically indicated for safety. **Checklist Review Interval:** Radiological tumor assessments via CT/MRI every 9 weeks.

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**15. Sign-off & Approval**

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|---------------------------------|--------|-----------|-------------|------|-------------|------|------|
| Role                            | Name   | Signature | Date        | :--- | :---        | :--- | :--- |
| <b>Principal Investigator</b>   | [Name] | _____     | 03-Apr-2026 |      |             |      |      |
| <b>Clinical Operations Lead</b> | [Name] | _____     | 03-Apr-2026 |      | <b>Lead</b> |      |      |
| <b>Statistician</b>             | [Name] | _____     | 03-Apr-2026 |      |             |      |      |